United Nations

World Health Organization

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Topic: The Question of the Implementation and Usage of Palliative Care instead of Curative Medications

Main Submitter: Japan, Germany

Co-Submitters: Belgium

Signatories: Australia, Spain, France, Sweden, Colombia, Poland, China, Portugal, South Africa, UK, Argentina, US, Norway, Thailand, Russia, India

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The World Health Organization, (Kenya, Netherlands, and Spain)

Recognizes the Sustainable Development Goals (SDGs), particularly Goal 3 – Good Health and Well-being –, which emphasizes the importance of ensuring a healthy life and promoting well-being for all;

Recognizes the importance of balancing palliative care with curative care. Along with curative care, palliative care prevents and relieves suffering through the early identification, correct assessment and treatment of pain and other problems, whether physical, psychosocial or spiritual.

Notes with concerns Worldwide, only about 14% of people who need palliative care currently receive it. While an estimated 56.8 million (including 25.7 million in the last year of life) people worldwide require palliative care, most of whom live in low- and middle-income countries . For children, 98% of those needing palliative care live in low- and middle-income countries with almost half of them living in Africa.

Recalling that over 56 million people worldwide currently require palliative care every year, foreseeing the increased need for integrating palliative care with the aging population, and the rise of noncommunicable and other chronic pain.

Grateful to the Universal Health Coverage (UHC) packages, and also (SDGs) goal 3.81 - Coverage of Essential health services -, which implies that all people receive the quality health services that they need, without the catastrophic health spending due to the healthcare costs.

Is aware of the advantages and importance of palliative care, which improves the quality of life of patients and that of their families who are facing challenges associated with life-threatening illness; The quality of life of caregivers improves as well.

Emphasizes the need for adequate funding mechanisms for palliative care programs, including for medicines and medical products, especially in developing countries.

Is aware of the National Institutes of Health (NIH)'s disadvantages in the focus of palliative medication. Evidence that progress has been extremely slow, and inequitable. Limiting the efficiency of funding. In low and middle-income countries (LMICS)

01. ***Calls for*** the creation of an organization – the IAPC (International Agency for Palliative Care) – for:

- ***a.*** Overseeing global implementation of palliative care strategies
- ***b.*** Forming a liaison between governments, research institutions, hospitals, the World Health Organization (WHO), and citizens;
- c. Encouraging HICs to form partnerships with LMICs to aid in resource mobilization, access, and funding for palliative care
- d. implementing laws that define the rank of priorities in terms of types of care, including a clear defining divide between curative medicine and palliative services. This will ensure that palliative care doesn't ever come in the way of curative and potentially life-saving treatment.

02. ***Promotes*** the effective integration of palliative care training into educational curricula,

- a. Integration into different levels of education, including but not limited to:
  - i. High school classes,
  - ii. Undergraduate degrees
  - iii. Medical schools,
  - iv. Postgraduate degrees,
  - v. Hospital training facilities,
  - vi. Institutional medical training,
- b. Promotion of various patient-centered approaches, such as but not limited to:
  - i. Habitual collection of patient data
  - ii. Requiring informed consent for treatments
  - iii. Initiation of collaboration between different healthcare professionals and social workers to address all of the patients' biopsychosocial needs,
- c. Spreading awareness through different platforms like:

i. Social media
ii. Government statements

03. ***Encouraging*** research into improved approaches to both palliative and curative care,

a. Requesting nations to follow a 75-25 framework for balancing research around curative and palliative treatments, respectively,
b. Research into alternative methods of pain relief other than opioids, especially for low to moderate pain relief,
c. Creating metrics that convey the efficiency of healthcare treatments and systems, such as but not limited to:

04. ***Vouching for*** the creation of the iAPC's Healthcare Platform (iAPC-HP) for the following purposes:

a. Connecting patients, family members, healthcare professionals, hospital management team, and researchers, for purposes such as but not limited to:
    i. End-to-end encrypted text, audio, and photo-based messaging,
    ii. Convenient reporting of symptoms,
    iii. Increasing convenience of requesting of home visits,
    iv. Regulating any inappropriate conduction of behavior by professionals by providing patients and their family members with an easy "report" button,
    v. Increasing transparency of costs,
    vi. Future-planning opportunities with physicians,
    vii. Technology-based tracking of health and social factors for care planning.
b. Providing a database of healthcare procedures, providing:
    i. A detailed and simple outline of procedures using visuals and audio to aid the understanding of the patient and their family members,
    ii. Average global costs of different treatments with and without health insurance,
c. Providing a "personal" tab with:
    i. Accessible patient history of medical procedures and past and chronic conditions,
    ii. Providence of their "will", consent forms, and dietary restrictions,
    iii. Uploading all data and files needing signatures to streamline legislative processes,
d. Providing a "public" tab for the raising of public awareness,
e. Calls for advertising of this platform in hospital settings and clinics, and for

05. ***Proposing*** the establishment of local healthcare networks with the incorporation of palliative care that :

a. Encourages mobile and accessible care to all areas of a country, specifically emphasizing rural areas,
b. Includes psychological care such as:
    i. Grief counselling
    ii. Psychotherapy
c. Takes into consideration all involved parties,
d. Helps keep palliative care, pain management techniques, and counselling culturally appropriate,
e. Encourages each city, district, or province to have its own network.

06. ***Urges*** member states to ensure access to palliative medication, including opioids, while maintaining strict safeguards against any misuse through:

a. The careful revision of current national regulations that unnecessarily restrict access to pain relief medications,
b. Encourages collaboration with international organizations to overcome shortages and logistical challenges,
c. Promote research into alternative methods of pain management that minimize the occurrence of withdrawal symptoms,

07. ***Demands*** Global Health Coverage (GHC) framework, which suggests that countries to:

a. Create universal health insurance with a copayment percentage equal to or below 50% to increase accessibility,
b. Fund for health coverage through methods provided in clause 9,

08. ***Develops*** partnerships between HICs and LMICs to provide resources to countries in need,

a. Essential medications such as;
    i. Opioids,
    ii. Ibuprofen,
    iii. Tylenol,
    iv. Anesthetics
    v. Antibiotic Ointments,

vi. Laxatives (For constipation),

b. Emergency kits that include:
   i. Defibrillators,
   ii. First Aid,
   iii. Food supplies
   iv. Flashlights,
   v. Water supplies,
   vi. Additional Batteries,

c. Diagnostic tools and machinery to reduce burdens on the healthcare systems, which include:
   i. Telemedicine
   ii. Artificial Intelligence beginning with for the purpose of radiology, and moving on other aspects of medicine

09. ***Proposes*** for the iAPC to receive funding for carrying out aforementioned tasks from the following sources:

a. Government subsidies,
b. Taxations that include:
   i. Alcohol,
   ii. Soft Drinks,
   iii. Tobacco,
   iv. Cigarettes,
   v. Casinos,
c. Funds collected by international organizations, such as the:
   i. WHO
   ii. World Bank
d. Allowing private investors to fund palliative care and preventative health programs.

10. ***Supports*** the use of verified messenger services to ship necessary medications especially to rural areas and LMICs, and ensure secure and authenticated communication between stakeholders;

11. ***Encourages*** the consensual implementation of curative care until :

a. The patient requests for it to stop
b. The patient experiences constant pain or side-effects for over 3 months with no signs of improvement
c. The hope for recovery is proven to be non-existent